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VERIDIAN INSTITUTE OF MEDICAL SCIENCES

Department of Radiodiagnosis & Imaging Sciences

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RADIOLOGY REPORT - CT THORAX (HRCT) & CHEST X-RAY PA

PATIENT NAME	Mrs. SUNITA KAPOOR	**MRN / PID**	VIMS-RD-7731902
AGE / GENDER	54 Y / Female	**ACCESSION NO.**	ACC-20261022-0457
REFERRING PHYSICIAN	Dr. R. Menon, Pulmonology	**EXAM DATE / TIME**	22-Oct-2026 / 10:15
WARD / BED	Medical Ward 3 / Bed 12	**REPORT DATE / TIME**	22-Oct-2026 / 13:40
MODALITY	DX (X-Ray), CT 128-slice	**CONTRAST**	Non-contrast

CLINICAL HISTORY

Fever with productive cough for 6 days, right-sided pleuritic chest pain. SpO2 91% on room air. Known type 2 diabetic. ? Community-acquired pneumonia. Rule out effusion / empyema.

COMPARISON

No prior imaging available for comparison.

TECHNIQUE

PA chest radiograph in erect position. Non-contrast volumetric CT of the thorax acquired from lung apices to the adrenal glands; 1 mm reconstructions in lung and mediastinal windows; multiplanar reformats reviewed. DLP: 342 mGy-cm.

FINDINGS

Chest radiograph: Homogeneous airspace opacity in the right lower zone with air bronchograms, obscuring the right hemidiaphragm. Blunting of the right costophrenic angle. Left lung is clear. Cardiac size is normal (CTR 0.46).

Lungs / airways: Dense consolidation involving the posterior and lateral basal segments of the right lower lobe with prominent air bronchograms. Adjacent ground-glass opacities and a few centrilobular tree-in-bud nodules in the right middle lobe, suggesting endobronchial spread. No cavitation. No evidence of pneumothorax. Left lung parenchyma is unremarkable. Central airways are patent.

Pleura: Small right-sided pleural effusion, maximum thickness 14 mm, simple density (approx. 8 HU). No pleural thickening or enhancement is assessable on this non-contrast study. No loculation.

Mediastinum / lymph nodes: Reactive right hilar and subcarinal lymph nodes, largest 12 x 9 mm (short axis). No pathologically enlarged mediastinal nodes. Heart size normal; no pericardial effusion. Thoracic aorta and main pulmonary artery are of normal calibre.

Upper abdomen / bones: Visualised upper abdomen shows mild hepatic steatosis. No focal lytic or sclerotic lesion in the visualised skeleton. Mild degenerative changes in the dorsal spine.

IMPRESSION

1. **Right lower lobe lobar pneumonia with air bronchograms, in keeping with the clinical suspicion of infective consolidation.**
2. **Associated small right parapneumonic effusion (14 mm), simple; no loculation. Clinical correlation / ultrasound-guided sampling only if it enlarges.**
3. **Tree-in-bud nodularity in the right middle lobe - likely endobronchial spread of infection.**
4. **Reactive right hilar and subcarinal lymph nodes. No mass lesion identified.**
5. **Follow-up chest radiograph in 6 weeks is advised to document complete resolution.**

Findings discussed with Dr. R. Menon on 22-Oct-2026 at 13:45.

SYNTHETIC SAMPLE - fictional patient, hospital and clinicians. Not a medical record. For software testing only.

Reported by:
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This is an electronically authenticated report. Radiological findings should be correlated clinically; imaging has inherent limitations. In case of discrepancy, please contact the department within 7 days.

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